FOUNDER CASE STUDY

Pranshakti Nutrition

Anubhav Kapoor, Co-founder

100+	₹2L	6–9 mo	₹8,999
CLIENTS SERVED	BEST MONTH, BOOKINGS	TYPICAL CLIENT RETENTION	BEST-SELLING PLAN (CORE)

I am the co-founder of Pranshakti Nutrition. My co-founder ran the nutrition and dietitian side of the business. I owned the product: pricing, the client discovery, the plan format, the decision to build the app, and the spec it was built from. Where I write “we” below, it is a call we made together. The business served 100+ clients between April 2023 and today, and hit about ₹2 lakh in bookings in its best month, built alongside a PGDM. This is the long version of that story: the decisions, the mistakes, and the real material behind each one.

01

SECTION 01 / 08

The problem

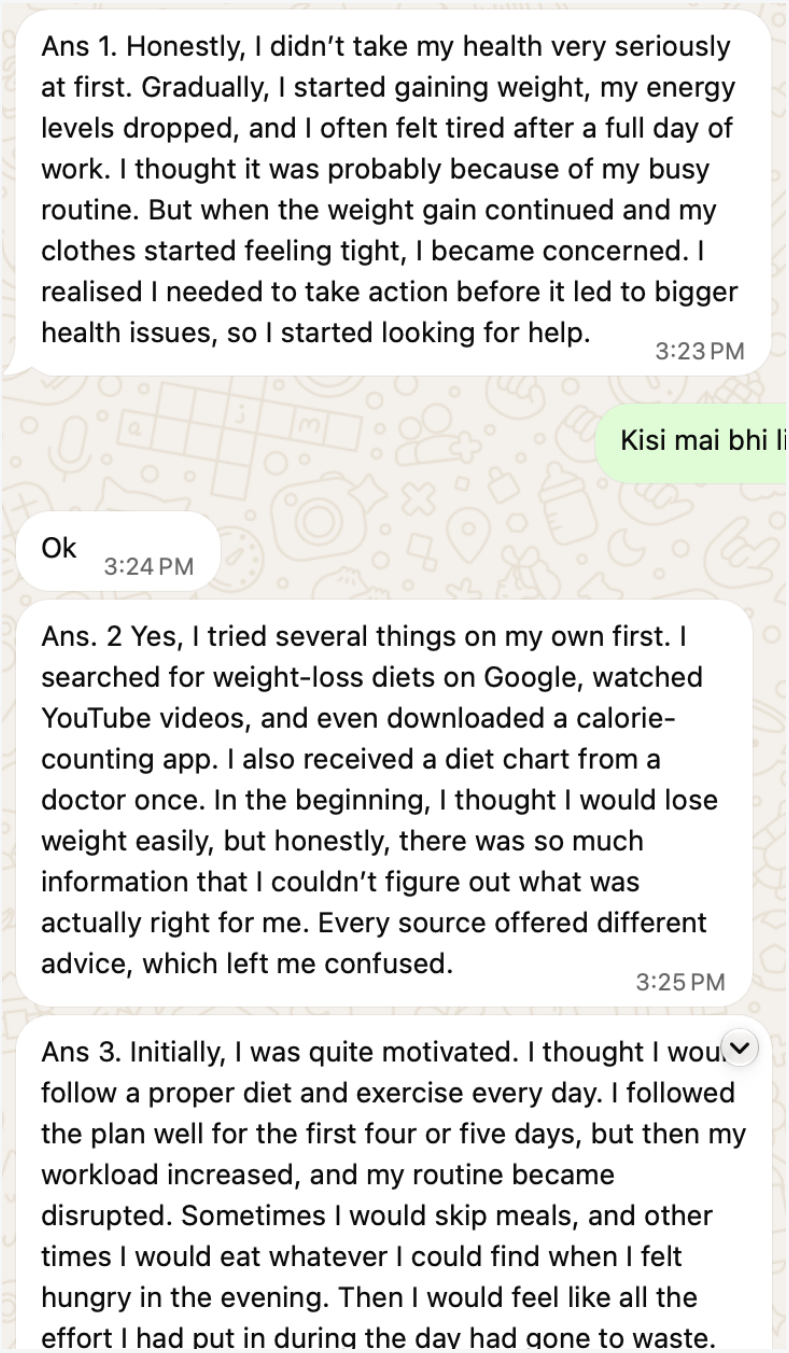
Most of our clients didn't ignore their health on purpose. The signs crept in slowly. One client, a 29-year-old working professional from a Tier 3 town in Haryana managing a thyroid condition, told me she didn't take her health very seriously at first. Weight gain, low energy, and tiredness after a full day of work built up gradually, until her clothes getting tight made it impossible to ignore. Like most people, she turned to the internet first: Google searches, YouTube videos, a calorie-counting app, and eventually a generic diet chart from a doctor. None of it fit how she actually ate. The chart leaned on oats and salads, while her everyday meals were roti, sabzi, dal and rice. She followed it for four or five days before her routine got disrupted, meals got skipped, and the plan quietly fell apart. What she needed was someone to notice when she slipped and help her get back on track.

That pattern repeated across almost every client we onboarded in the early months. People came to us with a drawer full of advice and no one checking in when real life got in the way. The gap was follow-through. Nothing we saw being sold in the market was built around that.

EXHIBIT

Client discovery, in her own words

Real WhatsApp answers from this client, collected before she started. No name shown.

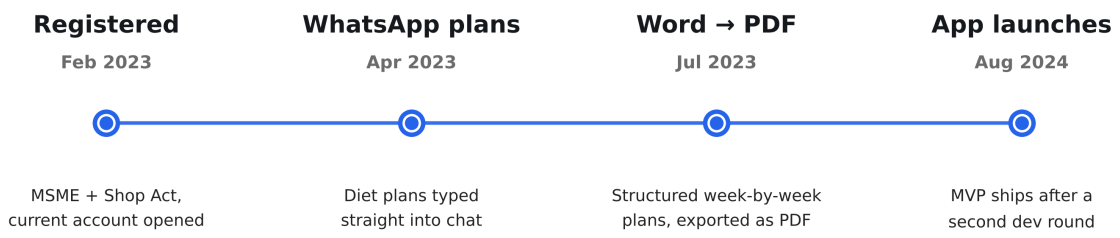


02

SECTION 02 / 08

What I did, in order

We registered Pranshakti Nutrition in February 2023, through MSME and the Shop Act, and opened a current account to actually take payments. Clients started coming in from April, and at first I gave diet plans straight over WhatsApp because it cost nothing extra and clients were already there. About two months in, that stopped being workable; typing a full plan into a chat window took too long and looked unstructured. I started looking for a better format, and by July 2023 I'd moved us to building each plan in Word, laid out week by week and day by day, with meal times written in, so we could actually structure a real plan instead of a loose message. Once a plan was built, we adjusted it for the individual client's food, timing and goals, exported it as a PDF, and sent that directly. The PDF became our standard delivery format for a long time, because it looked finished and clients could save or print it.



The sequence this section walks through, start to finish.

Even after we moved to Word and PDF, the delivery channel stayed WhatsApp, and so did the back-and-forth. A client would message asking for a change mid-week (a food they couldn't get, a meal that didn't fit their schedule) and we'd edit the plan and resend the whole thing. Editing and reconverting one client's plan took about 30 minutes on a clean day, and on a day with 8 to 10 clients needing changes, that alone was 4 to 5 hours. It usually wasn't clean. Word would mangle the font size or style on its own, and the meal table was fragile: a plan for a client with a specific condition could need a long instruction in one cell, like telling someone with a gastric issue to eat two wheat bran rotis with seasonal vegetables and buttermilk, then wait ten minutes and chew fennel seeds for digestion, and a line that long would break the table's formatting. Fixing that cost close to another hour a day on top of the editing itself. It worked, but it wasn't something that could keep scaling.

The jump to building an app came later, and it almost didn't happen. Before committing to anything, I looked at what competitor apps were already offering and which features actually mattered. We met several developers, and the real blocker was cost: building the app itself was one quote, and a proper admin panel that stayed in sync with it, which we needed to manage plans at scale, was another, much bigger one. I looked at cheaper developers too, but the language and approach they'd build in meant slower load and refresh times, and a slow app would have undone the exact problem we were trying to fix. So instead of cutting cost on the build itself, I cut scope: we paid the higher quote but built an MVP of only the top-priority features

rather than everything at once. Before committing even to that, I went back to clients and asked directly what they wanted the app to show, how they wanted to see their meals, and which features actually mattered to them, collected that into a list, and kept only the features that came up most. About eight or nine months after launch, once we could afford it, we paid for a second round of development and added the rest. One of the developers left partway through the first build for a medical issue, which pushed things back further, and the app wasn't actually ready until August 2024. Until then, every plan still went out as a Word-built PDF.

EXHIBIT

The actual Word-built, PDF-exported plan

A real client's day-by-day plan, laid out in Word and exported as a PDF exactly as described above, not a mockup. Name, age and weight blacked out for that client's privacy; the format and meals are real.



PRANSHAKTI NUTRITION

1

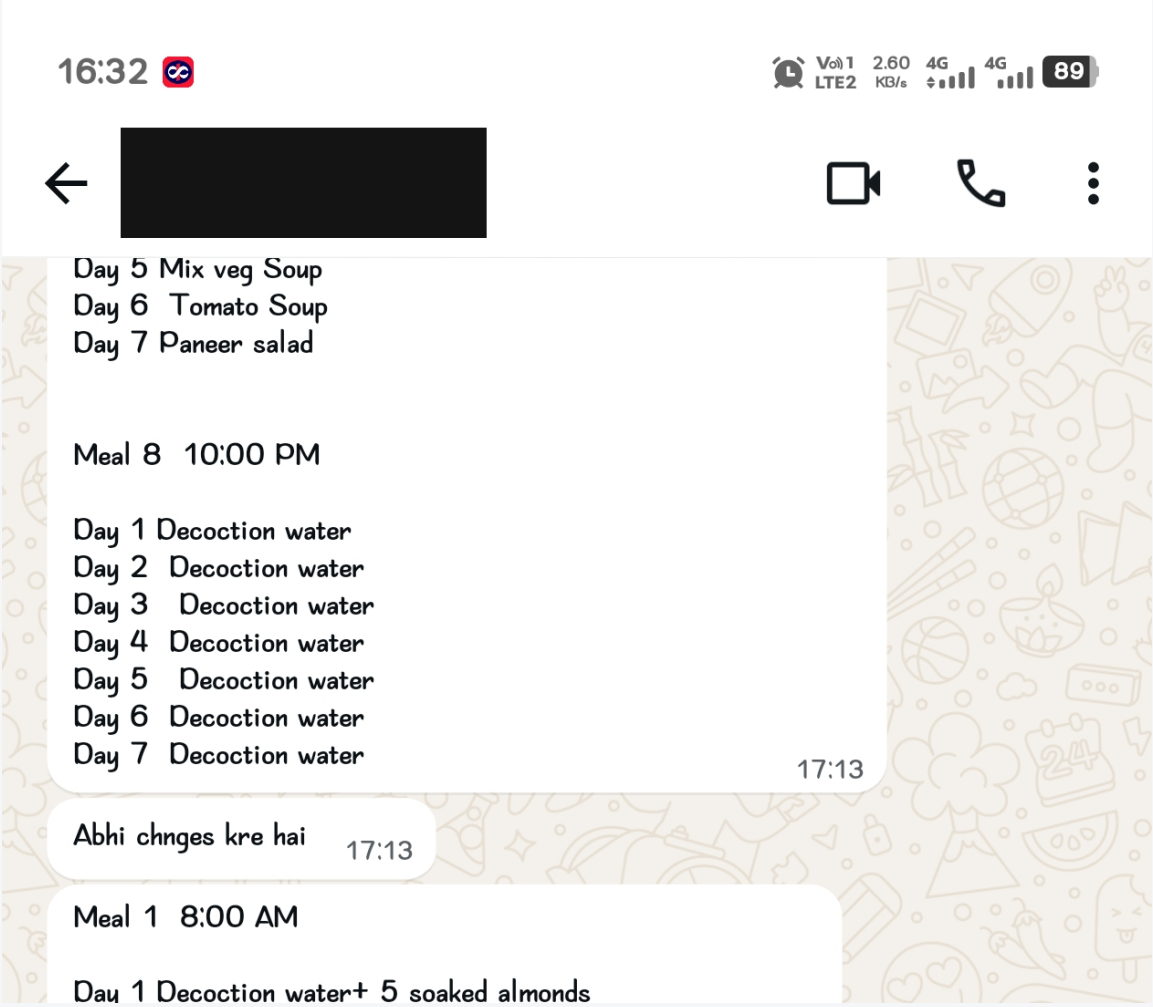
Name: [redacted]
Age- [redacted]
Weight: [redacted]
Diet Preference: vegetarian!

Date: 29-04-2024	Time	Food
Meal 1	8:00 AM	Elaichi water + overnight soaked 5 Almonds
Meal 2	9:30 AM	Veg oats
Meal 3	11:30 AM	Any fruit (seasonal) or chia seed water
Meal 4	2:00 PM	Any veg + 2 wheat roti with lassi
Meal 5	4:00 PM	Green tea
Meal 6	6:00 PM	Roasted murmura
Meal 7	8:00 PM	Bowl of Dal
Meal 8	10:00 PM	Elaichi water1 cup

EXHIBIT

The WhatsApp era, revised live

An actual plan sent over WhatsApp, with the client asking for changes mid-conversation. Name and photo blacked out for privacy.



SECTION 03 / 08

03 The pricing decision

I built pricing around a simple assumption: that clients would try the cheapest option first. I expected the 1-month Starter plan, at ₹2,999, to sell the most, since it let someone test whether Pranshakti actually delivered on what we promised before committing further. That's not what happened. Starter makes up only around a tenth of sales. The 90-day Core plan and the 6-month Transform plan do almost all the work instead, roughly 30% of sales each, with most Core buyers going on to extend into Transform or further once they've seen results.

The gap made sense once I looked closer. Clients did use the 1-month plan the way I expected, as a trial. But once they saw early results and trusted that we were doing what we said, they didn't renew into another 1-month plan. They converted straight into 3 or 6 months. The 1-month plan's real job was to convert trust into a bigger commitment, not to generate revenue on its own.

For the price floor, I worked backward from cost instead of guessing at what the market would pay. A monthly salary line plus our technology and platform costs came to about ₹2,15,000 a year: roughly ₹17,900 a month, about ₹600 a day, and close to ₹75 an hour across an 8-hour working day. ₹3,000 was the lowest monthly price that kept the business above that cost line. I also changed the exact price point twice over time, most notably to ₹2,999 instead of a flat ₹3,000, purely for how it read in marketing. The underlying math behind the floor never moved.

> *Its real job was to convert trust into a bigger commitment, not to generate revenue on its own.*

What the cost floor actually came from



₹3,000 was the lowest monthly price that stayed above this line

The same cost-per-year number above, run forward to a cost per hour.

Tier	Duration	Price	What's included	Share of sales
Starter	1 month	₹2,999	Diet plan, dietitian, app tracking	**~10%**
Core	3 months	₹8,999	+ metabolism plan, lifestyle coaching	**~30%**
Transform	6 months	₹11,999	+ maintenance month, yoga, pause option	**~30%**
Elite	9 months	₹15,999	+ immunity focus, priority support	**~20%**
Pinnacle	12 months	₹17,999	+ a full year of support	**~10%**

SECTION 04 / 08

04 Acquisition

Three channels brought in clients: word of mouth and existing-client referrals, hospital tie-ups, and Instagram. Ranked by volume, referrals from clients who had already seen results were the largest source, hospital tie-ups were second, and Instagram was last, despite getting the most of our attention and time.

Ranked by quality, the order held: referrals first, hospital tie-ups second, Instagram last. That's worth pausing on, because volume and quality rarely line up this cleanly. A referred client arrived

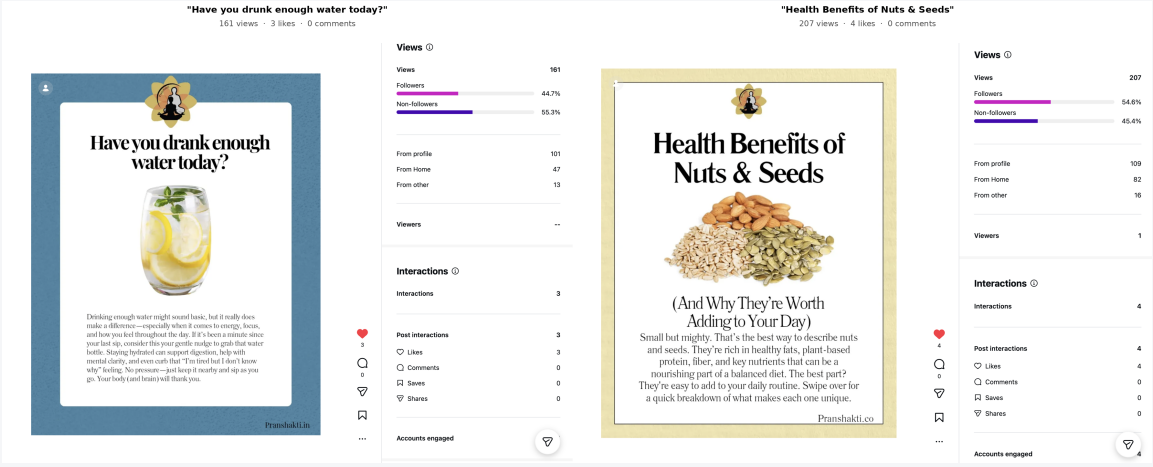
already trusting us, because someone they knew had lost weight through Pranshakti. To make that referral worth acting on, we built a bonus into it, and it evolved as we learned. At first both sides got 15 days added to their plan for free. Later we changed the referrer's side to a choice, ₹1,000 cash or 15 days, whichever they preferred, and kept 15 days plus an extra 5% discount for the person being referred. That's a real, calculable cost: on our most common plan, the 90-day Core at ₹8,999, the referrer's side runs ₹1,000 to ₹1,500 and the referee's 5% discount adds roughly another ₹450, so a referral costs us somewhere around ₹1,500 to ₹2,000 against a blended average order value across our tiers of roughly ₹11,500, well under a fifth of what it brings in. That's exactly why it turned satisfied clients into a direct, low-cost acquisition channel, and it converted at a higher rate than anything else we tried. Hospital tie-ups worked on different economics: I used personal connections to get in the door, and paid the doctor a 30% incentive on whatever a referred client spent. A doctor's referral carried trust the client didn't have to build with us from scratch. Two hospital tie-ups brought in 10 clients between them, a small number, but every one of them came in pre-sold.

Instagram never earned that same trust. There was no consistent face behind the account and no budget for content, so it stayed generic, and generic content does not convert the way a personal referral does. A typical post reached 150 to 200 accounts and drew one to four interactions total, almost all of them likes, with zero comments or saves. We kept posting through most of the company's life, telling ourselves that consistency would eventually pay off. Once we actually compared the numbers, referrals and hospital tie-ups were converting at a far higher rate for a fraction of the effort, and we leaned into both rather than continuing to prop up Instagram.

EXHIBIT

Instagram Insights, two typical posts

Not cherry-picked: two consecutive posts from @pranshakti.co, screenshotted directly from Instagram's own Insights panel. Reach and engagement both stayed in this range across the account's life.



05

SECTION 05 / 08

Retention

The first thing we tried for retention didn't work, and it took us a while to admit that. We sent clients reminders and check-in messages on WhatsApp, the same channel we used to deliver their plan. Clients ignored most of them. WhatsApp was already full of messages from everyone else in their life, and ours just blended into that noise, read at best, often not opened at all. We assumed the problem was what we were saying, so for a while we tried rewording the reminders rather than questioning the channel itself.

What actually fixed it was moving check-ins and updates into the app once we built one. The same kind of nudge that got ignored on WhatsApp got opened inside the app, consistently. A WhatsApp message competes with a hundred other things on a client's phone that day; something inside an app a client chose to open doesn't get lost the same way.

Clients who stayed did so by renewing into a new plan rather than buying one long plan upfront. Most stayed for six to nine months, and the reason was a good one: that's roughly how long it took someone with a straightforward weight goal to get there. A client who hit their target and stopped checking in hadn't churned, we'd finished helping them. The clients who engaged with their plan, whether on WhatsApp or later in the app, were the ones who renewed, though I should be careful here: engaged clients renewing may just mean motivated clients do both. What would actually settle it is whether a client who was drifting could be pulled back by a nudge in the app, and I never ran that test.

The exception, and the longest single engagement we had, was our very first client, managing a thyroid condition and high blood pressure after moving to Australia on a work reference. His case was less about weight and more about lifestyle and allergies layered on top of a chronic condition, made harder by living alone and an awkward time difference with us. He started on a 12-month plan. Over the first eight months, closely following it, his TSH came down enough that his thyroid medication dropped from 150mg to 112mg alongside real weight loss. A trip home to see his parents broke the routine; his diet slipped, the weight came back, and his dosage went back up. We pitched a 6-month extension suited to where he actually was, he took it, and over that second stretch he lost roughly 10 kg and his medication came down to 100mg. End to end, that's about 18 months, and it's the clearest evidence we have that the product works for something harder than weight loss alone, not a number we'd claim as a company-wide average.

> *The problem was never what we were saying. It was the channel.*

06

SECTION 06 / 08

The product, in detail: the Home screen

The app replaced something specific: the back-and-forth of a PDF plan that a client had to keep opening, scrolling through, and cross-checking against whatever else they were tracking. I wrote the spec for this screen directly from the client discovery above. The Home screen became the single place a client needed. It greets them by name, shows weight, calories and water at a glance, and lists today's meals with times and a one-tap way to mark each one done, with the full week available behind “View All.” Below that sits water intake against a daily goal, with quick-add buttons for common amounts, then a streak counter and badges to give clients a small reason to come back daily. The only things that take a client elsewhere are entering their weight and editing their profile.

The first version wasn't so much broken as unspecified. The developer started from her own default: a generic stock-photo banner and a “tips to lose weight” widget, the same for every client, on a product whose entire premise was that generic advice doesn't work. I looked at it and said, in effect, okay, let's go with that, without pushing back or specifying what it should actually be instead. That's on me, not on her. The meal list below it also often showed placeholder entries instead of the client's actual food, and weight and progress were buried further down the screen than a client would ever look first. It took calling a round of clients for direct feedback, plus what I was picking up in my PGDM about customer experience and product thinking, before I actually designed the screen properly myself: what a client should see first, where their real meals and weight belonged, and what could wait behind a tap. Only after that redesign did the generic banner and tips widget come out. On top of the design problem, the build also had real bugs, unrelated to any of this: meals got cut off or misaligned, the layout jumbled in places, the font didn't render consistently, the app loaded slowly, and it sometimes threw an error on refresh. None of these were small annoyances. Together they made the one screen meant to replace a PDF feel less reliable than the PDF had been.

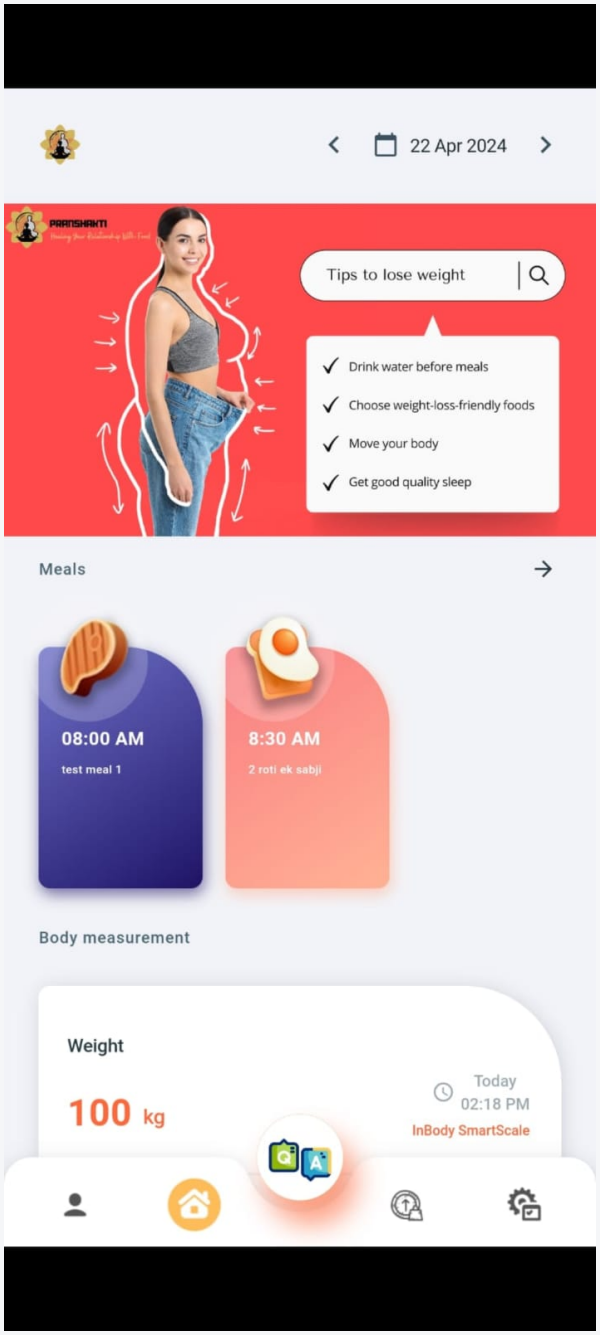
I fixed the defects separately from the redesign: correcting the meal layout and alignment, fixing the font rendering, and addressing the load time and refresh errors. Clients now describe it as smooth, with everything visible clearly and no slowdown.

One thing worth addressing directly: this screen has a streak counter and badges, the same mechanic I flagged as manipulative when I reviewed Swiggy's app elsewhere in this portfolio, where gamifying the moment right after a health-conscious order works against the user it's meant to serve. The mechanic is the same here, but the job isn't. Swiggy's scratch card fires after someone has completed their goal and pulls them toward another order they didn't plan. The streak here fires before the behaviour and rewards doing the thing the client is already paying us to help them do. One manufactures a reason to come back, the other reflects a commitment the client already made. I know that's a fine line, and the same argument gets made in bad faith all the time, so the honest test is whether the mechanic survives the client hitting their goal: if the streak keeps them logging in after the plan ends, it's become engagement for its own sake, and it should go.

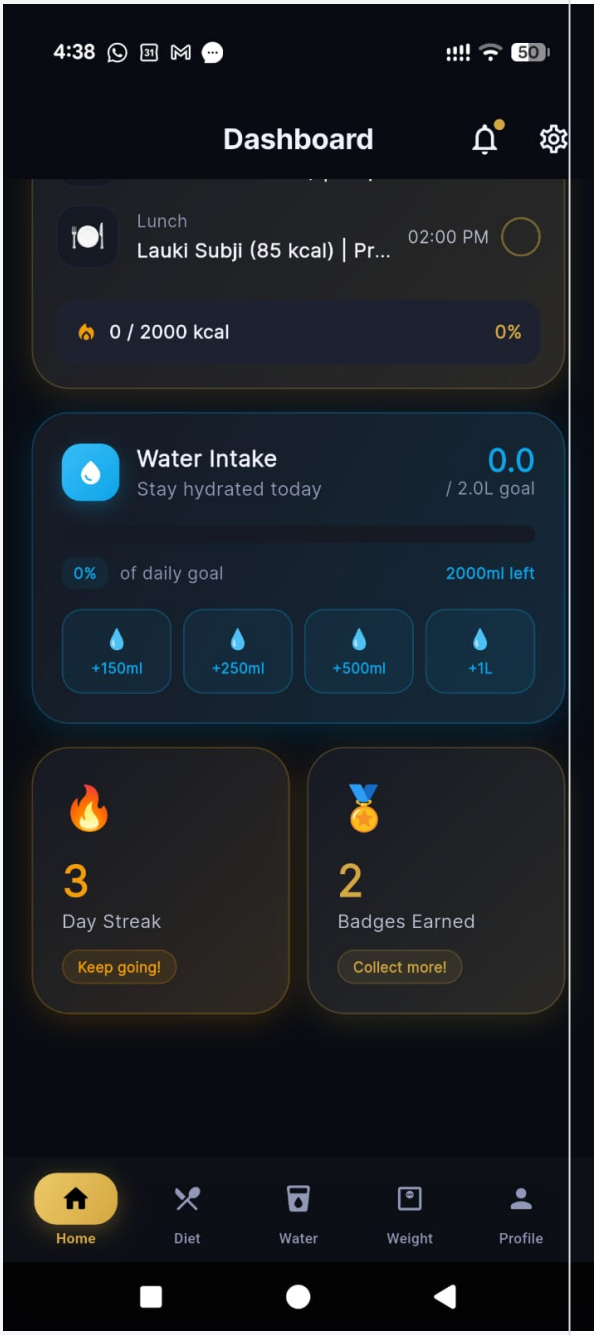
EXHIBIT

The Home screen, before and after

From a generic stock-photo banner and placeholder meals to the real dashboard described above.



First version



Current version

07

SECTION 07 / 08

What I got wrong

The active client base today is well below its peak, and that's the most important thing to say plainly here. Acquisition didn't stop outright; it just never got systematised. Once my PGDM got demanding enough that I couldn't stay as hands-on, we hired a dietitian and brought in people to run Instagram and paid ads. I agreed to spend on ads, a few thousand rupees worth, but the people running it didn't have experience in the health space and couldn't find a content formula that actually made the posts and reels engage; we parted ways after about three months. We looked at proper marketing agencies too, but the ones we found had fixed packages well outside what a bootstrapped business our size could afford, so rather than force that spend, we kept putting the capital we had into the tech, to keep giving clients a better experience. The channels that worked early, referrals and hospital tie-ups, never got the same attention once I stepped back, and nobody else owned growth the way I had.

The clearer personal misjudgment was in how I handled the dietitian hire. She was based in Hyderabad, and she started well. But being a two-person startup, we had no system to track her client calls, and she was using her personal number to reach clients directly. I should have built that oversight in from day one and didn't. Months in, a client told us she'd offered to be their dietitian independently, at a 20% discount, if they dropped their plan with Pranshakti. We confronted her and she resigned. The business survived it because the client told us rather than taking the offer, which is luck I shouldn't have been relying on.

The 12-month plan is a smaller version of a lesson the pricing data already told us. Clients earned our trust in 90-day increments, not in one leap, and asking someone to commit to a full year upfront, before they had any proof we could deliver, was too big an ask. Only a small fraction of clients ever took it.

And for close to a year and a half, we ran the entire client relationship over WhatsApp before building anything more structured. That delayed every lesson in this case study by roughly that much: the channel problem behind retention, the real cost of manual plan-editing, all of it was sitting there the whole time. We just hadn't built a way to see it clearly yet.

> *Clients earned our trust in 90-day increments, not in one leap.*

08

SECTION 08 / 08

What I would do next

Right now, Pranshakti works. The product is personalised, retention holds up through renewals, and the client base has proven that people will pay and stay. The gap is acquisition, for the reasons in the previous section, and it's the one I'd fix first. I'm not prioritising product or retention work below because the evidence says those hold up; the gap is entirely at the top of the funnel.

With a team and a budget, I would do three things, in this order.

First, I would turn the client-referral program into the main acquisition channel instead of a side incentive. It already converts better than anything else, because a client arrives already trusting us. I would put a small budget behind making it easy to refer, rather than relying on word of mouth alone, since referrals are the lowest-cost, highest-quality channel we have and the easiest one to scale first. Concretely: one person owning it part-time, a monthly budget in the ₹15,000-25,000 range to fund the reward and an in-app prompt at the moment a client hits a result, and a 90-day target of roughly doubling referral-driven enquiries off their current run rate. If they haven't grown at least 40% by then, that tells me the incentive or the ask needs rework, not more time.

Second, I would turn the hospital tie-ups into a repeatable playbook and go find more of them. They converted clients who were already pre-sold, for the same reason referrals worked, but we only ever built two of them, both through personal connections. A documented process for approaching a hospital, and someone dedicated to running it, would turn a channel that worked by accident into one that works on purpose. Concretely: one person spending part of their week on outreach, a simple one-page pitch and follow-up sequence, and a 90-day target of one new signed tie-up with at least three more in active conversation. If we're not talking to at least three hospitals by day 60, the pitch or the targeting needs to change.

Third, I would build real visibility into the funnel, from enquiry to first plan to renewal, instead of judging each channel by instinct. We knew Instagram wasn't converting because it felt that way, not because we were tracking it. I would put this third, after the two channels we already know work, because the priority is scaling what's proven before spending more to diagnose what isn't. Concretely: a 2-3 week setup, no more than a small freelance budget, and a single success metric: weekly conversion numbers by channel, in one place, so the next spend decision is made on data instead of instinct.

01

Referral program

Turn it into the main channel

OWNER
One person, part-time

BUDGET
₹15,000–25,000/mo

90-DAY TARGET
~2x referral enquiries

IF IT MISSES
<40% growth → rework the ask

02

Hospital tie-ups

Make it a repeatable playbook

OWNER
One person, part of week

BUDGET
Near-zero — pitch + follow-up

90-DAY TARGET
1 new tie-up, 3+ in talks

IF IT MISSES
<3 by day 60 → change targeting

03

Funnel visibility

See conversion, stop guessing

OWNER
External freelancer, one-time setup

BUDGET
Small freelance budget, ~2–3 weeks

90-DAY TARGET
Weekly channel-level conversion

IF IT MISSES
— (this just needs to exist)

The three moves above, with the owner, budget and 90-day target that decides whether to keep going.

GET IN TOUCH

Happy to walk through any part of this in more detail.

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